This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practise under it.

Healthcare professionals must work within their own competence. This PGD does not remove professional obligations or accountability.

ISSUED, VALID FROM 30 SEPTEMBER 2026

Patient Group Direction

for the supply of Minoxidil 2.5 mg tablets (Loniten), off-label at low dose for the treatment of Androgenetic Alopecia (Male and Female Pattern Hair Loss)

Androgenetic Alopecia (Male and Female Pattern Hair Loss)

Adults aged 18 to 65 years, men and women.

Patient Group Direction, version 002, issued 30 September 2026. Minoxidil 2.5 mg tablets (Loniten), off-label, at 1.25 mg to 5 mg daily. Adults aged 18 to 65 with androgenetic alopecia, men and women. Pharmacists only.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

What this PGD is for

The supply of low-dose oral minoxidil to an adult aged 18 to 65 with androgenetic alopecia (male or female pattern hair loss), by a registered pharmacist in a community pharmacy, face to face on the premises, without a prescription. Supply under this PGD is a private service and is OFF-LABEL: minoxidil tablets are licensed for severe hypertension only.

What it authorises

- Minoxidil 2.5 mg tablets (Loniten) at a starting dose of 2.5 mg once daily for men and 1.25 mg (half a tablet) once daily for women, for a first supply of 8 weeks.
- Continuation supplies of 8 weeks after a recorded review at each supply, with the dose unchanged, reduced once for a non-cardiovascular side effect, or, once only and not before the 24 week review, increased to 5 mg daily (two tablets) for men or 2.5 mg daily for women.
- A maximum of six 8-week supplies under this PGD, counted by supply number from the first supply and not reset by any gap or restart. Further supply requires a documented prescriber review as defined under Dose and frequency, after which a further period of up to six 8-week supplies may follow under this PGD; the dose increase is available once in the patient's whole course under this PGD.

What it does not cover

- Anyone under 18 or over 65, at any supply. A patient who turns 66 finishes the 8-week supply in hand and is then referred to a prescriber. The SmPC advises a lower starting dose in patients over 65 because of orthostatic hypotension, and the 2024 consensus reached no agreement on adolescents.
- Hair loss that is not androgenetic: sudden, patchy, scarring, inflamed or painful scalp, heavy diffuse shedding, frontal fibrosing alopecia, or hair loss with hyperandrogenism or polycystic ovary syndrome in a woman. Refer.
- Anyone with cardiovascular disease, hypertension whether or not treated, hypotension or orthostatic intolerance, a history of pericarditis or pericardial effusion, phaeochromocytoma, renal or hepatic impairment, or who takes any medicine that lowers blood pressure or raises the heart rate. The exclusion criteria list these in full.
- Pregnancy, breastfeeding, planning a pregnancy, or a woman of childbearing potential who is not using contraception.
- Doses above 5 mg daily for men or 2.5 mg daily for women, twice daily dosing, the 5 mg or 10 mg tablets, or any use for blood pressure.

- Supply by a pharmacy technician, or any remote or telephone consultation. The scalp is examined and the blood pressure measured on the premises by the pharmacist who supplies.
- A patient not registered with a GP, since GP notification is a condition of supply.

Summary of the governing guidance for low-dose oral minoxidil in pattern hair loss

Loniten 2.5 mg tablets SmPC (Pfizer Limited, PL 00057/1006, text revised February 2026); Akiska Y et al., Low-dose oral minoxidil initiation for patients with hair loss: an international modified Delphi consensus statement, JAMA Dermatology, online 20 November 2024; Vano-Galvan S et al., Safety of low-dose oral minoxidil for hair loss: a multicenter study of 1,404 patients, J Am Acad Dermatol 2021;84:1644-51; Penha MA et al., Oral minoxidil vs topical minoxidil for male androgenetic alopecia: a randomized clinical trial, JAMA Dermatology 2024;160:600-5; NICE Clinical Knowledge Summary, Alopecia, androgenetic (male and female), current version. Summarised 28 September 2026.

The condition

Androgenetic alopecia (pattern hair loss) is progressive miniaturisation of scalp hair follicles under the influence of androgens in genetically susceptible people. In men it presents as bitemporal recession and vertex thinning (Hamilton-Norwood pattern); in women as diffuse thinning over the crown with preservation of the frontal hairline (Ludwig pattern). Onset is gradual, over months to years, without scalp inflammation, scarring or patches of complete loss.

NICE CKS: the diagnosis is clinical. Consider another cause where hair loss is sudden, patchy, diffuse with heavy shedding, scarring, itchy or painful, or where a woman has signs of hyperandrogenism (hirsutism, acne, irregular periods). In women with diffuse loss CKS suggests checking ferritin and thyroid function, because iron deficiency and thyroid disease can cause or worsen shedding.

Licensed treatments in the UK are topical minoxidil (2% and 5%, pharmacy medicines for men and women) and oral finasteride 1 mg (men only, prescription only; covered by the Get Real Health Male Pattern Hair Loss PGD). Oral minoxidil has no UK licence for hair loss.

Oral minoxidil: the licence and the off-label position

Loniten (minoxidil) tablets are licensed only for severe hypertension, at 5 mg to 100 mg daily, and at those doses must be given with a diuretic and a beta-blocker because of salt and water retention and reflex tachycardia. The SmPC (section 4.4) records pericardial effusion in about 3 to 5 per cent of treated patients not on dialysis, hypertrichosis in most patients, ECG T wave changes in about 60 per cent, and (section 4.2) advises a 2.5 mg starting dose over 65 because of orthostatic hypotension, all in the context of antihypertensive doses.

Use for hair loss is at a fraction of those doses (0.25 mg to 5 mg daily) and is OFF-LABEL. The international consensus (43 dermatologists, 12 countries, 2024) supports low-dose oral minoxidil as a treatment for androgenetic alopecia in adults, with the most frequently used starting doses of 2.5 mg daily for men and 1.25 mg daily for women, adjusted afterwards for response and tolerance.

The 2021 multicentre series of 1,404 patients on 0.25 mg to 5 mg daily found hypertrichosis in 15.1 per cent, lightheadedness in 1.7 per cent, fluid retention in 1.3 per cent, tachycardia in 0.9 per cent, headache in 0.4 per cent, periorbital oedema in 0.3 per cent and insomnia in 0.2 per cent; 1.7 per cent stopped because of side effects. No pericardial effusion was reported in that series. Women were more likely than men to report hypertrichosis.

The 2024 randomised trial in men found oral minoxidil 5 mg daily and topical 5% twice daily produced similar increases in hair density at 24 weeks, with more hypertrichosis and headache in the oral group.

Who the consensus recommends it for, and who it excludes

Adults aged 18 and over with androgenetic alopecia who have not responded to, cannot tolerate, or do not wish to use topical minoxidil, or who prefer an oral treatment. It may be used alone or alongside finasteride in men.

CONTRAINDICATIONS. In the SmPC: hypersensitivity to minoxidil or any excipient, and phaeochromocytoma; the SmPC also states minoxidil is not recommended in pregnancy or in women of childbearing potential not using contraception (neonatal hypertrichosis has been reported), and that it is excreted in milk. The consensus adds pregnancy and breastfeeding, and a history of pericardial effusion or pericarditis, as contraindications to low-dose use for hair loss.

PRECAUTIONS: cardiovascular disease of any kind, including heart failure, ischaemic heart disease, arrhythmia and valvular disease; hypotension or a history of syncope; renal impairment or dialysis; hepatic impairment; concurrent antihypertensive treatment, which is additive; older age, where the SmPC advises a lower starting dose because of orthostatic hypotension.

BASELINE: blood pressure and heart rate, a cardiovascular history, current medicines, and for women a pregnancy and contraception check. Routine baseline ECG or blood tests are not required by the consensus in healthy adults.

MONITORING: review at 3 to 6 months for response, blood pressure, heart rate, weight and side effects, then at least annually. The patient must be told to stop and seek advice for chest pain, palpitations, breathlessness, ankle swelling or fainting.

What to tell the patient

Response takes time: increased shedding is common in the first 4 to 8 weeks as miniaturised hairs are replaced, visible improvement takes 3 to 6 months, and the effect is lost within months of stopping. Treatment is long term.

Hypertrichosis (extra hair on the face, arms and body) is the commonest side effect and matters more to women; it is dose related and reverses within months of stopping.

Fluid retention (ankle swelling, weight gain), dizziness on standing, a fast heartbeat and headache can occur and should be reported. Chest pain, breathlessness or fainting mean stop and seek medical help.

Women must not become pregnant while taking it, and must stop before trying to conceive.

The Patient Group Direction

How to use this PGD

This is an off-label supply, and the document says so everywhere it matters

- MINOXIDIL TABLETS ARE LICENSED FOR SEVERE HYPERTENSION ONLY, at 5 mg to 100 mg daily with a diuretic and a beta-blocker. Use for hair loss at 1.25 mg to 5 mg daily, taken alone, is off-label. No UK national guideline recommends it; NICE CKS records it only as a specialist option. The basis for authorising it under this PGD is the practice of dermatologists set out in the 2024 international consensus statement, the published safety series of 1,404 patients and the 2024 randomised trial, summarised in part 2, which the authorising signatories accept as current best practice for this condition. NICE MPG2 permits off-label use under a PGD where it is justified by best clinical practice and the PGD says so. Adopting pharmacies should note that the men's 5 mg step equals the licensed starting dose for hypertension; what differs is that it is taken without a diuretic or beta-blocker, as the consensus describes, in people who have been screened to have a normal blood pressure and no heart disease.
- THE PATIENT MUST BE TOLD IT IS OFF-LABEL, why, and what the licence is for, and must consent on that basis. The consent script under Counselling is the minimum, the patient signs the consent statement on the consultation record, and the record says so.
- THE RISK STATEMENT, in one place. At the doses used for hair loss, the 1,404 patient series found hypertrichosis in 15 per cent and a cardiovascular or fluid effect (lightheadedness, fluid retention or tachycardia) in about 4 per cent, with 1.7 per cent stopping for side effects and no pericardial effusion. The SmPC warnings about pericardial effusion (3 to 5 per cent), salt and water retention and ECG change come from hypertensive patients on 5 mg to 100 mg daily. This PGD screens out heart disease, hypertension and the medicines that add most to the effect, measures blood pressure, pulse and weight at every supply, caps the dose, and tells the patient the symptoms to stop for. No ECG is required. No blood test is required for men; women need the ferritin and thyroid results described below because those conditions mimic the female pattern.

Who this is for

- Adults aged 18 to 65 with a clinical diagnosis of androgenetic alopecia: gradual thinning over months to years in the typical male pattern (temples and crown) or female pattern (crown, frontal hairline kept), with a normal, non-inflamed, non-scarred scalp, examined on the premises with the hair parted.
- Any other pattern is not this PGD. Sudden loss, patches, scarring, itch, pain, scale, broken hairs, heavy shedding, a receding frontal hairline in a woman with loss of the eyebrows (frontal fibrosing alopecia), or a woman with hirsutism, acne, irregular periods or polycystic ovary syndrome: refer to the GP. Appendix 2 lists what to look for.
- WOMEN need a ferritin of 30 micrograms/L or more and a thyroid stimulating hormone within the laboratory range, tested within the last 12 months, before a first supply, because iron deficiency and thyroid disease cause diffuse hair loss that looks like the female pattern and are treated differently. The result may be from the GP (NHS App or letter) or from a private laboratory; either is acceptable if the pharmacist sees it. A woman without results is advised to ask her GP or to arrange a private test, and to return with them; she is not supplied today. A ferritin below 30 is referred to the GP.
- It may be used with topical minoxidil, or in men alongside finasteride under the Male Pattern Hair Loss PGD. Tell the patient that the oral and topical products are the same drug and that stopping the topical product once the oral is established is reasonable.

The numbers

- MEN: 2.5 mg once daily. WOMEN: 1.25 mg once daily, half a 2.5 mg tablet. One step up in the whole course, not before the fourth supply visit and only where the response is inadequate against the baseline photographs, the patient has had no stop symptom and no hypertrichosis they mind, today's systolic is 100 or more and not more than 10 mmHg below the baseline reading (the lower seated systolic recorded at the first supply, or at a restart), and the pulse is within the limits: men to 5 mg (two tablets), women to 2.5 mg. Never more, never twice daily. One step down at any review for hypertrichosis, headache or another side effect that is not cardiovascular; women on 1.25 mg cannot step down and stop instead.
- BLOOD PRESSURE AND PULSE AT EVERY SUPPLY, seated and standing. Seated blood pressure after 5 minutes rest, two readings 1 to 2 minutes apart with the lower recorded: systolic 100 to 139 mmHg and diastolic below 90 mmHg (systolic 90 to 99 is acceptable only where the patient has no dizziness or faintness). Standing blood pressure after 1 minute: no fall of 20 mmHg or more in systolic or 10 mmHg or more in diastolic, and no symptoms. Resting pulse 50 to 100. Where a reading is outside these limits, a third seated reading after a

further 5 minutes ("on repeat") decides, and a patient outside the limits on repeat may return once on another day.

- WEIGHT AT EVERY SUPPLY on the pharmacy scales, in light clothing; height and body mass index at the first supply. ONE RULE FOR FLUID AND BLOOD PRESSURE SYMPTOMS: a weight gain of 2 kg or more since the previous supply, new ankle or facial swelling, dizziness or faintness, palpitations, chest pain or breathlessness means STOP, refer (same day for chest pain, breathlessness, palpitations, fainting or facial swelling; otherwise routine GP within a week), and no further supply under this PGD until a prescriber has reviewed the patient and confirmed in writing that treatment may restart. Dose reduction is not an option for these symptoms.
- EVERY SUPPLY IS 8 WEEKS (56 days) and is numbered, with a review at each. Supply visit 4 (about week 24 when the patient attends on time) is a full review with repeat photographs. At supply visit 6 the pharmacist repeats the photographs and the full review and sends that record for the prescriber review, which must be complete before a seventh supply. MAXIMUM SIX SUPPLIES under this PGD, counted by supply number from the first supply and not reset by a gap or restart; the calendar date does not matter, the count does.
- A gap of more than 4 weeks without tablets is a restart at the starting dose, with the scalp re-examined and a new baseline blood pressure. A restart does not reset the supply count. A gap caused only by waiting for the prescriber review after supply 6 is not a restart provided the patient has been off tablets for no more than 8 weeks; tell the patient to hold their last week of tablets until the review is confirmed.
- Response takes 3 to 6 months. Shedding in the first 4 to 8 weeks, settling by 12 weeks, is expected and is not a reason to stop; heavy shedding that persists beyond 12 weeks is a reason to stop and refer for diagnosis. The benefit is lost within months of stopping.

Stop and seek help

- Tell every patient, and write it on the label and the record they take away: STOP THE TABLETS AND SEEK MEDICAL HELP THE SAME DAY for chest pain, breathlessness, palpitations, fainting, or swelling of the face; and STOP AND CONTACT THE PHARMACY OR GP WITHIN A WEEK for ankle swelling, dizziness or faintness, or a weight gain of 2 kg or more. Stopping for any of these is a cardiovascular stop: no further supply under this PGD without a prescriber review.

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Patient Group Direction

for the supply, for the treatment of Androgenetic Alopecia (Male and Female Pattern Hair Loss), of:

Minoxidil 2.5 mg tablets, low dose

Patient Group Direction for the off-label supply of minoxidil 2.5 mg tablets (Loniten) at 1.25 mg to 5 mg daily for androgenetic alopecia in adults aged 18 to 65 years.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC. This PGD is not used by pharmacy technicians: the diagnosis, the cardiovascular screen and the off-label consent are the pharmacist's.
- All users must have completed the Get Real Health training module for this PGD and passed its competency assessment before first supply, and must be familiar with the current Loniten SmPC, the BNF entry for minoxidil, and the 2024 international consensus summarised in part 2.
- All users must understand what off-label supply under a PGD means, and must be able to explain it to a patient in plain language and record the consent.
- ALL USERS MUST BE ABLE TO RECOGNISE THE FEATURES IN APPENDIX 2 that distinguish androgenetic alopecia from alopecia areata, telogen effluvium, scarring alopecia including frontal fibrosing alopecia, tinea capitis and traction alopecia, and must know that every one of those is a referral, not a supply.
- All users must be competent in measuring seated and standing blood pressure and pulse with a validated automatic monitor, and in applying the limits in this PGD.
- All users must know the cardiovascular warnings in the SmPC (salt and water retention, tachycardia, pericarditis and pericardial effusion) and the symptoms the patient must be told to stop for.
- All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required. All users must work in compliance with the SOPs of their own employer, and must practise only within the bounds of their own competence. Indemnity must cover an off-label supply under this PGD.
- All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so.
- All users must be familiar with the current Summary of Product Characteristics for every product named in this PGD, and with current BNF and national guidance for the condition.
- All users must be up to date with MHRA drug safety alerts and recalls relevant to these products.
- All users must be up to date with their CPD requirements and appraisal.
- All users must hold appropriate professional indemnity covering this service.
- All users must understand capacity and consent, including the Mental Capacity Act 2005, and must be able to assess consent in children and young people where this PGD covers them.

The PGD

Indication	Androgenetic alopecia (male pattern or female pattern hair loss) in an adult aged 18 to 65 who wishes to treat it and in whom the diagnosis is clinically clear, where an oral treatment is preferred, or topical minoxidil has not helped or is not tolerated. OFF-LABEL: minoxidil tablets are licensed for severe hypertension only; use for hair loss is authorised under this PGD on the basis of the guidance summarised in part 2. Supply under this PGD is a private

	service, face to face on the pharmacy premises.
Inclusion criteria	● **Aged 18 to 65 years inclusive at every supply. A patient who turns 66 during treatment finishes the supply in hand and is then referred to a prescriber; no further supply under this PGD.** ● **A clinical diagnosis of androgenetic alopecia made on examination of the scalp today: gradual onset over months or years, in the male pattern (bitemporal recession, vertex thinning) or the female pattern (diffuse thinning over the crown with the frontal hairline preserved), with a normal scalp (no redness, scale, scarring, pustules, pain or itch), no patches of complete loss, and fewer than 6 hairs released by a gentle pull on a bundle of about 50 to 60 hairs at the first supply. Baseline photographs taken and stored in the record with the patient's consent: the crown from above and the frontal hairline from the front, hair parted in the midline, same room lighting, to be repeated in the same way at supply visits 4 and 6.** ● **Seated blood pressure after 5 minutes rest, two readings 1 to 2 minutes apart with the lower recorded: systolic 100 to 139 mmHg and diastolic below 90 mmHg (systolic 90 to 99 is acceptable only where the patient has no dizziness or faintness). Standing blood pressure after 1 minute: no fall of 20 mmHg or more in systolic or 10 mmHg or more in diastolic, and no symptoms. Resting pulse 50 to 100. Where a reading is outside these limits, a third seated reading after a further 5 minutes ("on repeat") decides, and a patient outside the limits on repeat may return once on another day.** ● **For a woman: a ferritin of 30 micrograms/L or more and a thyroid stimulating hormone within the laboratory reference range, tested within the last 12 months by the NHS or a private laboratory, seen by the pharmacist (NHS App, GP letter or laboratory report) and recorded with the values and dates.** ● **For a woman of childbearing potential (any woman who is not post-menopausal, defined as 12 months without periods after the age of 45 with no other cause, and not permanently sterilised; absence of periods because of contraception, breastfeeding or recent childbirth does not remove childbearing potential): not pregnant on her own account with the last menstrual period recorded, not breastfeeding, not planning a pregnancy during treatment, and using contraception she agrees to continue and to tell the pharmacy if she stops. Accepted methods: any hormonal method, an intrauterine device or system, sterilisation or a vasectomised partner, or consistent condom use where she declines another method. Fertility awareness or withdrawal alone are not accepted. Record the method.** ● **The patient is registered with a GP and agrees to the GP being informed of the supply, of any dose change and of any stop for a side effect, within 7 days. A patient who will not allow the GP to be told is not supplied.** ● For a continuation supply: fewer than six supplies under this PGD since the first (or since the last prescriber review), no gap of more than 4 weeks without tablets (or, after supply 6, no more than 8 weeks while awaiting the prescriber review), today's blood pressure, pulse and weight within the limits, and none of the stop symptoms since the last visit. ● For a restart after a gap of more than 4 weeks: the scalp re-examined and the pull test repeated as at a first supply, the medicines and cardiovascular history re-taken, a new baseline blood pressure recorded, the starting dose supplied, and the supply count continued from where it was. ● **Valid informed consent obtained, including explicit consent to**

	off-label treatment after the explanation under Counselling, with the consent statement on the consultation record signed by the patient. ● The patient understands this is a private service and what it costs. ● No exclusion criterion present.
Exclusion criteria	**Refer, do not supply, where any of the following applies.** ● Under 18 or over 65 years of age at the time of supply. ● Confirmed hypersensitivity to minoxidil (oral or topical) or to any excipient (lactose monohydrate, microcrystalline cellulose, starch, colloidal silicon dioxide, magnesium stearate); rare hereditary galactose intolerance, total lactase deficiency or glucose-galactose malabsorption. ● **Hair loss that is not clearly androgenetic (see Appendix 2): sudden onset; patchy loss; scalp redness, scale, pustules, scarring, pain or itch; loss of follicular openings; a receding frontal or temporal hairline in a woman, or loss of the eyebrows (frontal fibrosing alopecia); broken hairs; at the first supply, 6 or more hairs on the pull test or heavy diffuse shedding (the expected shedding in the first 12 weeks of treatment is not an exclusion at review; heavy shedding persisting beyond 12 weeks of treatment is, and is referred for diagnosis); hair loss that began or worsened within 12 months of childbirth, a serious illness, major surgery, rapid weight loss or starting a medicine known to cause hair loss; or in a woman, hirsutism, new acne, irregular periods, a deepening voice, or a diagnosis of polycystic ovary syndrome. Refer to the GP.** ● A woman without ferritin and thyroid results from the last 12 months, a ferritin below 30 micrograms/L, a thyroid stimulating hormone outside the reference range, or a thyroid or iron problem under investigation or not yet stable on treatment. ● **ANY CARDIOVASCULAR DISEASE: ischaemic heart disease or angina, previous myocardial infarction, heart failure, any arrhythmia including atrial fibrillation, valvular heart disease, cardiomyopathy, congenital heart disease, previous stroke or transient ischaemic attack, peripheral arterial disease, pulmonary hypertension, postural tachycardia syndrome or other orthostatic intolerance, or a history of pericarditis, pericardial effusion or pleural effusion.** ● **HYPERTENSION, whether treated, untreated or managed by lifestyle; or a reading today of 140 mmHg systolic or more, or 90 mmHg diastolic or more, on the lower of two seated readings and again on a third reading after a further 5 minutes seated ("on repeat" throughout this PGD means that third reading). A patient outside the limits on repeat at a first visit may return once on another day; outside the limits again, they are referred to the GP for blood pressure assessment and not supplied. HYPOTENSION, with the same one return visit: on repeat, systolic below 90 mmHg, or systolic 90 to 99 with dizziness or faintness; on the single standing reading, a fall of 20 mmHg or more in systolic or 10 mmHg or more in diastolic, or symptoms on standing. Unexplained syncope at any time, or any syncope in the last 12 months. Resting pulse below 50 or above 100 on repeat.** ● **ANY OF THE FOLLOWING MEDICINES, for any indication: any antihypertensive; any beta-blocker including propranolol for anxiety or migraine; any calcium channel blocker; any diuretic including spironolactone; an alpha-blocker (tamsulosin, doxazosin, alfuzosin, prazosin); a nitrate; sacubitril/valsartan; clonidine, moxonidine or methyldopa; daily tadalafil or any other phosphodiesterase-5 inhibitor taken daily; an SGLT2 inhibitor**

	(dapagliflozin, empagliflozin, canagliflozin, ertugliflozin); a stimulant (methylphenidate, lisdexamfetamine, dexamfetamine, modafinil), regular pseudoephedrine or phenylephrine, or non-prescribed stimulant drug use. The effect on blood pressure or heart rate is additive and the SmPC warns of excessive hypotension. Also a systemic corticosteroid, because of salt and water retention. Medicines not in this list are dealt with under Cautions. ● Phaeochromocytoma. ● Renal impairment (known eGFR below 60, chronic kidney disease stage 3 or worse, or dialysis), or hepatic impairment. The SmPC requires attention to salt and water balance in renal impairment and has no data in hepatic impairment. ● Known anaemia not yet treated, any thyroid disease that is not stable on treatment, an eating disorder, or a body mass index below 18.5 (height and weight measured at the first supply). ● **Pregnancy, breastfeeding, planning a pregnancy within the treatment period, or a woman of childbearing potential not using contraception. Minoxidil is not recommended in pregnancy; neonatal hypertrichosis has been reported after exposure; it is excreted in milk.** ● Already taking oral minoxidil from another source; or a course under this PGD stopped for a cardiovascular stop (a weight gain of 2 kg or more, ankle or facial swelling, dizziness or faintness, palpitations, chest pain, breathlessness, fainting, or blood pressure or pulse outside the limits) without a prescriber review since that confirms in writing that treatment may restart. ● **At a continuation visit (not at a restart, where the weight and symptom comparison starts afresh from today's values), a CARDIOVASCULAR STOP: weight gain of 2 kg or more since the previous supply, new ankle or facial swelling, palpitations, chest pain, breathlessness, dizziness, faintness or fainting since the last visit, or blood pressure or pulse outside the limits above on repeat. Stop and refer: same day for chest pain, breathlessness, palpitations, fainting or facial swelling; otherwise routine GP within a week. No dose reduction and no further supply until a prescriber review.** ● Six supplies under this PGD completed without a documented prescriber review since. ● The patient does not accept off-label treatment after the explanation, is not registered with a GP or will not allow the GP to be informed, or lacks capacity to consent.
Cautions	**Hypertrichosis. Growth of fine hair on the face, arms and body is the commonest side effect (about 15 per cent at low dose; most patients at antihypertensive doses). It is dose related and reverses within 1 to 6 months of stopping. Every patient must be warned before the first supply; it matters more to women, and a woman who would find facial hair unacceptable may reasonably decline. At review it is a reason to step the dose down where the patient is above the starting dose, or to stop where a woman is on 1.25 mg; it is not a cardiovascular stop and the patient may restart later.** Phosphodiesterase-5 inhibitors (sildenafil, tadalafil, vardenafil) taken on demand: the blood pressure lowering effects add together and minoxidil's effect lasts for days, so there is no safe interval to advise. Tell the patient that light-headedness after the combination is possible, to sit or lie down if it happens, and not to take a second PDE5 dose that day. Daily tadalafil is an exclusion. Erectile dysfunction in a man under 40 can be an early sign of vascular disease; ask about exercise tolerance and chest symptoms and, where there is any doubt, refer before supplying.

	Alcohol and hot environments add to vasodilation. Advise moderation in the first weeks and standing up slowly. Men taking finasteride, whether under the Male Pattern Hair Loss PGD or prescribed elsewhere, and anyone using topical minoxidil: concurrent use is permitted. Record the other treatment and its source; where both are supplied here, review them at the same visits. Tell the patient the oral and topical products are the same drug and that stopping the topical product once the oral is established is reasonable. Diabetes on metformin, sulfonylureas, DPP-4 inhibitors, GLP-1 agonists or insulin, migraine, asthma, well-controlled epilepsy, stable treated hypothyroidism and depression are not exclusions in themselves. Medicines that can lower blood pressure or cause fluid retention and are not in the exclusion list (a daily NSAID, a tricyclic antidepressant, a phenothiazine, pregabalin or gabapentin): measure the blood pressure with that in mind, warn about dizziness and swelling, and refer where in doubt. Contraception. The suitability of a woman's contraceptive method is her prescriber's responsibility, but where something obvious is wrong (for example combined hormonal contraception with migraine with aura) do not make a first supply until her prescriber has reviewed the method and she attends with the outcome; at a continuation visit, supply may continue on her existing method provided she is seen about it within 4 weeks and the pharmacist records the referral. If her method is stopped or changed she must tell the pharmacy and confirm the replacement method before she continues minoxidil; without contraception she stops minoxidil. Weight loss treatment (GLP-1 agonists, orlistat, dieting). Not an exclusion, but ongoing weight loss can mask the 2 kg fluid retention trigger; ask specifically about ankle swelling, tight shoes or rings and breathlessness at every supply, and treat new swelling as a cardiovascular stop whatever the scales say. Lactose. Each tablet contains 95.8 mg lactose monohydrate. Not relevant in ordinary lactose intolerance at this quantity; relevant only in the rare hereditary disorders listed under Exclusions. Driving. Dizziness can occur, particularly at the start. Advise the patient not to drive if affected. Surgery or a new cardiovascular diagnosis. Tell the patient to tell any doctor or anaesthetist that they take minoxidil, and to stop and seek advice if a heart or blood pressure medicine is started.
Actions if the patient is excluded or declines	● Explain the reason. Where the exclusion is a blood pressure reading, say so plainly and advise the patient to have it assessed by their GP whatever they decide about hair loss: routinely for a reading of 140/90 to 179/119; the same day for 180/120 or above, or for chest pain, breathlessness, headache with visual change or confusion. ● **Offer the licensed alternatives: topical minoxidil 5% (pharmacy medicine, men and women), and for men finasteride under the Male Pattern Hair Loss PGD where its criteria are met. Record which was offered.** ● Where the hair loss pattern is not androgenetic, refer to the GP for diagnosis rather than offering any treatment; scarring alopecia, alopecia areata and tinea capitis need a diagnosis first. ● Where a woman has no ferritin and thyroid results, advise her to ask her GP for them or to arrange a private test, and to return with the results; a ferritin below 30 is referred to the GP for treatment first. ● **Where a patient on treatment is excluded at review for a side effect, tell them to stop, give the stop-and-seek-help advice in writing, and refer: the same day for chest pain, breathlessness, palpitations, fainting or facial swelling; otherwise to the GP within a week.** ● Document the reason, the advice given and the decision reached. Inform or refer to the GP as appropriate.

Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP as appropriate. Because this is an off-label use, report every cardiovascular adverse event, however minor.
Records to be kept	• **That valid informed consent was given, and specifically that the patient was told the supply is off-label, what the licence is for, why it is being used, and the alternatives offered, and signed the consent statement on the consultation record.** • Patient name, address, date of birth, sex, and the GP with whom they are registered, and that the GP was informed within 7 days of the first supply, of any dose change and of any stop for a side effect. • **The pattern and duration of hair loss, the features checked on examination (scalp normal, no patches, no scarring, hairline, eyebrows, pull test), and that baseline photographs were taken and stored.** • **Cardiovascular history and current medicines, including that no medicine in the exclusion list is taken.** • **Seated blood pressure (both readings and any repeat), standing blood pressure, pulse and weight measured today, with the values; height and body mass index at the first supply.** • **For a woman: the ferritin and thyroid results seen and their dates; pregnancy excluded and last menstrual period; breastfeeding excluded; and the contraceptive method.** • **The product supplied with its PL number, the dose (1.25 mg, 2.5 mg or 5 mg daily), the number of tablets, the batch number and expiry date, and whether this is a first supply, a continuation or a restart, with the date of the first supply and the number of supplies so far under this PGD (maximum six between prescriber reviews).** • Where the dose was changed: the previous dose, the review findings that justified the change, and that the patient was told. • **That the hypertrichosis warning and the stop-and-seek-help advice were given, in writing.** • The date the next review is due, and that the patient was told; at supply visits 4 and 6, that repeat photographs were taken and the response recorded; and after six supplies, the prescriber review held. • Name and registration number of the pharmacist supplying. • Advice given, including advice given if the patient is excluded or declines. • Details of any adverse drug reactions and the actions taken. Records signed, dated, legible and contemporaneous, kept for 8 years. The patient must be given a written record of the dose, the stop-and-seek-help advice and the review date.

The medicine

Name, form and strength	• MINOXIDIL 2.5 mg TABLETS (Loniten, Pfizer Limited), PL 00057/1006, round white to light tan biconvex tablets, scored, which can be divided into equal halves. Blister packs of 60 and bottles of 100. Only this product is supplied under this PGD; record the product and PL number supplied. • **The 5 mg and 10 mg tablets are NOT used under this PGD. The men's 5 mg dose is two 2.5 mg tablets.** • Excipients: lactose monohydrate 95.8 mg per 2.5 mg tablet, microcrystalline cellulose, starch, colloidal silicon dioxide, magnesium stearate.
Legal category	POM.

Dose and frequency	• **OFF-LABEL. Licensed use is 5 mg to 100 mg daily for severe hypertension with a diuretic and a beta-blocker. The doses below are for hair loss, are taken alone, and are the starting doses most often used in the 2024 consensus; the one-step titration, the review schedule and the stop rules are the rules of this PGD.** • **MEN: 2.5 mg (one tablet) once daily, at the same time each day, with or without food.** • **WOMEN: 1.25 mg (half a tablet) once daily. Show the patient how to halve the tablet along the score line; a tablet splitter may be supplied. The other half is kept in the original blister or container and taken within 24 hours.** • FIRST SUPPLY: 8 weeks (56 days). Review at 8 weeks, before the supply runs out: blood pressure, pulse, weight, side effects and adherence. Response is not assessed at this review. • CONTINUATION: supplies of 8 weeks (56 days) at weeks 8, 16, 24, 32 and 40, with a review at every supply recording blood pressure (seated, standing and any repeat), pulse and weight, and a full review with repeat photographs and a response assessment at supply visit 4 and again at supply visit 6, the latter forming the record sent for the prescriber review. • **DOSE INCREASE: once only, at or after the fourth supply visit, where the response is inadequate against the baseline photographs, the patient has tolerated the starting dose without any stop symptom or hypertrichosis they mind, and today's systolic is 100 mmHg or more and not more than 10 mmHg below the baseline reading (the lower seated systolic at the first supply, or at the most recent restart), with pulse within the limits: men to 5 mg (two tablets) once daily, women to 2.5 mg (one tablet) once daily. The increase is available once in the patient's whole course under this PGD; after a restart at the starting dose, a patient who had already used it may resume the higher dose at or after the fourth supply visit following the restart if the same conditions are met, but no patient has two increases.** • **DOSE REDUCTION: at any review, for hypertrichosis, headache, insomnia, gastrointestinal upset or another side effect the patient finds unacceptable that is NOT a stop symptom, reduce to the starting dose (men 2.5 mg, women 1.25 mg). Where the starting dose is not tolerated, stop; that is not a cardiovascular stop and the patient may try the starting dose once more at a later date if they wish, after which the PGD does not apply. Dizziness, faintness, swelling, weight gain of 2 kg or more, palpitations, chest pain and breathlessness are never managed by dose reduction: they are cardiovascular stops.** • **MAXIMUM DURATION: six 8-week supplies (48 weeks) under this PGD from the first supply, not reset by a gap or restart. PRESCRIBER REVIEW: before any seventh supply, a prescriber (the patient's GP, or where the GP declines, a Get Real Health prescriber by face to face or video consultation) reviews the blood pressure, pulse and weight record, the photographs and the side effects, and confirms in writing that treatment may continue. Before any restart after a cardiovascular stop, the review must be face to face (the GP, or a Get Real Health prescriber in person) with examination for oedema and a repeat blood pressure, and the written confirmation must say the symptom has resolved off treatment; the restart is then at the starting dose whatever the dose before the stop, and the tablets are not resumed in the meantime. The pharmacist holds the written confirmation in the record. A further period of six supplies may then follow under this PGD with the same pattern: full review at the fourth and sixth visits of that period and the next prescriber review sought at its sixth visit. The review is**

	sought at supply visit 6 so that it is complete before the tablets run out. ● RESTART: after a gap of more than 4 weeks without tablets, start again at the starting dose with the restart inclusion criteria met. A restart counts towards the six supplies and does not reset the count. ● MISSED DOSE: take it when remembered the same day; do not double up. ● STOPPING: may be stopped at any time without tapering. Hair regained is lost over 3 to 6 months.
Quantity to be supplied	Every supply is 8 weeks (56 days): 56 tablets of 2.5 mg for a man at 2.5 mg, or 112 tablets at 5 mg; 28 tablets of 2.5 mg for a woman at 1.25 mg (halved), or 56 tablets at 2.5 mg. Supply as a dispensed medicine in a suitably labelled container with the manufacturer's patient information leaflet.
Route and method of administration	Oral. Swallow with water. The 2.5 mg tablet is halved along the score line for the 1.25 mg dose.
Labelling	Label as a dispensed medicine with the patient's name, the dose in plain words ("ONE tablet once a day", "TWO tablets once a day" or "HALF a tablet once a day"), the date and the pharmacy, and add: "Stop and seek medical help the same day for chest pain, breathlessness, palpitations, fainting or swelling of the face; stop and contact the pharmacy or GP within a week for ankle swelling, dizziness or a weight gain of 2 kg or more." Supply the manufacturer's patient information leaflet, and tell the patient that it describes use for blood pressure because that is the licence.
Storage	Store below 25 C in the original package to protect from moisture. Bottles tightly closed.
Adverse effects	At the low doses used for hair loss (multicentre series, 1,404 patients): hypertrichosis 15 per cent, lightheadedness 1.7 per cent, fluid retention 1.3 per cent, tachycardia 0.9 per cent, headache 0.4 per cent, periorbital oedema 0.3 per cent, insomnia 0.2 per cent. From the SmPC at antihypertensive doses, and to be watched for at any dose: very common tachycardia, pericarditis, hypertrichosis and hair colour change, ECG T wave change; common fluid retention, oedema, pericardial effusion and tamponade, gastrointestinal upset; uncommon pleural effusion, breast tenderness; rare leucopenia, thrombocytopenia, Stevens-Johnson syndrome, bullous dermatitis, rash; frequency not known angina, toxic epidermal necrolysis, peripheral oedema, raised creatinine and urea. Increased shedding in the first 4 to 8 weeks is an expected drug-induced effect and is not a reason to stop or to report.

Information for the patient

Written information	Supply the patient information leaflet for the product, and a written record of the dose, the stop-and-seek-help advice, the hypertrichosis warning and the review date. Explain that the leaflet describes the licensed use for blood pressure.
Counselling	● **OFF-LABEL CONSENT SCRIPT, to be given in substance and recorded: "Minoxidil tablets are licensed in the UK to treat severe high blood pressure, at higher doses than this and alongside other heart medicines. Using a low dose on its own for hair loss is not covered by the licence and no UK guideline recommends it yet. Skin specialists use it this way; an international expert group published guidance on it in 2024, and a study of 1,400 patients found it generally well tolerated, with extra body hair the commonest side effect. Because it lowers blood pressure, we check your blood pressure, pulse and weight at every visit, there are heart symptoms you must stop for, and after six supplies**

	(about eleven months) a doctor must review you before we can continue. The licensed alternatives are minoxidil lotion or foam on the scalp and, for men, finasteride tablets. Are you happy to go ahead on that basis?" The patient signs the consent statement on the record. ● Take one dose a day at the same time. Women: half a tablet, split along the line; keep the other half for tomorrow. ● **You may shed more hair in the first 4 to 8 weeks. That is expected. Improvement takes 3 to 6 months, and if you stop, the gain is lost over a few months.** ● **Extra fine hair on the face, arms or body is the commonest side effect, and the colour of your hair can change. Both go away over a few months if you stop, and where you are above the starting dose it can be lowered. Stop and seek advice for a rash or blistering of the skin or mouth.** ● **STOP THE TABLETS AND SEEK MEDICAL HELP THE SAME DAY for chest pain, breathlessness, a racing or irregular heartbeat, fainting, or swelling of the face. STOP AND CONTACT US OR YOUR GP WITHIN A WEEK for swollen ankles, dizziness or faintness, or if you put on 2 kg or more. If you stop for any of these, a doctor must see you before we can supply again.** ● Men: if you use sildenafil, tadalafil or a similar tablet, you may feel light-headed after taking it with minoxidil; sit or lie down until it passes and do not take a second dose that day. Tell us if you start buying it over the counter. ● Stand up slowly, go easy on alcohol in the first few weeks, and do not drive if you feel dizzy. ● **Women: you must not become pregnant while taking this. Keep using your contraception, tell us if you stop it, and stop the tablets before trying for a baby.** ● Tell any doctor, dentist or pharmacist you see that you take minoxidil, especially if you are given a blood pressure or heart medicine. We will let your GP know. ● Come back at your review date, before your tablets run out; a gap of more than 4 weeks means starting again at the first dose. We check blood pressure, pulse and weight before each supply, and after six supplies a doctor reviews you before we can continue; keep your last week of tablets until that review is confirmed.
Follow-up	Review in the pharmacy at every 8 week supply, with a full review and repeat photographs at supply visits 4 and 6: blood pressure (seated, standing and any repeat), pulse, weight, side effects, adherence and response. Inform the GP within 7 days of the first supply, of any dose change and of any stop for a side effect. After six supplies, obtain the written prescriber review before any further supply. Refer to the GP the same day for chest pain, breathlessness, palpitations, fainting or facial swelling, or a blood pressure of 180 systolic or 120 diastolic or above (this PGD's own threshold); within a week for ankle swelling, a weight gain of 2 kg or more, dizziness or faintness, or a reading of 140 systolic or 90 diastolic or above on repeat. A patient mid-course when this version expires continues under the successor version, or is referred to a prescriber if there is none.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions.
- Loniten 2.5 mg tablets SmPC (Pfizer Limited, PL 00057/1006, text revised February 2026); Akiska Y et al., Low-dose oral minoxidil initiation for patients with hair loss: an international modified Delphi consensus statement, JAMA Dermatology, online 20 November 2024; Vano-Galvan S et al., Safety of low-dose oral minoxidil for hair loss: a multicenter study of 1,404 patients, J Am Acad Dermatol 2021;84:1644-51; Penha MA et al., Oral minoxidil vs topical minoxidil for male androgenetic alopecia: a randomized clinical trial, JAMA

Dermatology 2024;160:600-5; NICE Clinical Knowledge Summary, Alopecia, androgenetic (male and female), current version. Summarised 28 September 2026.

- The current Summary of Product Characteristics for each product named in this PGD, medicines.org.uk.
- British National Formulary, current edition.

Authorisation of this version

Authorised by the Medical Director and the Head Pharmacist named below. This version is not valid without both signatures.

Version	v002
Supersedes	None (new PGD)
Valid from date	30 September 2026
Expiry date	31 July 2027
Review date	31 May 2027, or earlier if the Loniten SmPC changes or UK guidance on oral minoxidil for hair loss is published

Authorised on behalf of Get Real Health by the Medical Director and the Head Pharmacist. Their signatures for this version are on the signed authorisation page at the end of this document.	

PGD adoption and authorisation by the employer or clinical lead

The employer has ensured that the person using this PGD has the knowledge and skills to safely provide the service, and that appropriate governance is in place. This section must be signed by the superintendent or clinical lead responsible for this service.

To be completed by the adopting pharmacy. These signatories are the adopting organisation own signatories and must not be pre-filled by Get Real Health.

Superintendent / clinical lead	Name: ..... Job title and organisation: ..... Signature: ..... Date: .....
Professional group signatory	Name: ..... Job title and organisation: ..... Signature: ..... Date: .....

Agreement to practise by the registered healthcare professional

By signing below you confirm that you agree to the contents of this PGD and that you will work within it. A PGD does not remove the inherent professional obligations or accountability of the individual practitioner. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of the pharmacy or clinic premises to which this PGD relates

Premises name	.....
Address	.... Postcode: .....
ODS code, if applicable	.....

Registered healthcare professionals authorised to work under this PGD at those premises

The employing organisation must keep this page, or an equivalent local register, and must be able to produce it on request. A practitioner who has not signed against the current version is not authorised to work under it.

Name of healthcare professional	Job title | Registration number | Signature | Date
.....	..... | ..... | ..... | .....
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.....	..... | ..... | ..... | .....
Valid from date	30 September 2026
Expiry date	31 July 2027

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version	v001
Date	28 September 2026
Changes	● New PGD. Low-dose oral minoxidil for androgenetic alopecia in men and women, agreed by the Medical Director and the Head Pharmacist. It replaces nothing: the earlier catalogue entry "Female Pattern Hair Loss, Minoxidil" was withdrawn on 11 September 2026 because no minoxidil PGD existed behind it. Developed by the Medical Director (a GP) and the Head Pharmacist from the sources in Appendix 1, without specialist dermatology or cardiology input. ● The supply is off-label throughout. The licence (severe hypertension, 5 mg to 100 mg daily with a diuretic and a beta-blocker), the absence of a UK guideline, and the basis for off-label use (the 2024 international consensus, the 2021 safety series of 1,404 patients, the 2024 randomised trial, NICE MPG2) are stated on the cover, in the indication, in the dose row, in the counselling script and in the records, and the patient signs the consent statement. ● Pharmacists only. The diagnosis of androgenetic alopecia against the differential in Appendix 2, the cardiovascular screen and the off-label consent are not delegated to pharmacy technicians, and the consultation is face to face on the premises. ● Doses follow the consensus starting doses: 2.5 mg daily for men and 1.25 mg daily for women. The one-step increase (men 5 mg, women 2.5 mg) is permitted only at or after the 24 week review against baseline photographs with a systolic floor; a one-step reduction is permitted for non-cardiovascular side effects only. The 5 mg and 10 mg tablets are not used. Twice daily dosing and doses above those stated are outside this PGD. ● Every supply is 8 weeks and numbered, so that the rules run on the supply count rather than the calendar: full review at supply visit 4, prescriber review sought at supply visit 6, maximum six supplies under this PGD not reset by a gap or restart, then a documented prescriber review before any further supply, to meet Schedule 16 and MPG2 on long-term treatment. A gap of more than 4 weeks is a restart at the starting dose with its own inclusion criteria; a restart after a cardiovascular stop needs a face to face prescriber review. ● Ages 18 to 65 at every supply: the SmPC advises a lower starting dose over 65 because of orthostatic hypotension and the consensus reached no agreement on adolescents. A patient who turns 66 finishes the supply in hand and is referred. ● Exclusions are cardiovascular by design: any cardiovascular disease including orthostatic intolerance, hypertension whether or not treated, a blood pressure of 140/90 or above or below 90 systolic today, a postural fall, syncope, a pulse outside 50 to 100, every class of blood pressure lowering or heart rate raising medicine named, phaeochromocytoma, renal or hepatic impairment, untreated anaemia, unstable thyroid disease, an eating disorder or low body mass index, and pregnancy, breastfeeding or absent contraception in a woman of childbearing potential. ● Blood pressure is measured twice seated and once standing, with pulse and weight, at every supply, with stated limits, a defined third reading on repeat and one return visit; height and body mass index at the first supply. One rule for fluid and blood pressure symptoms: a weight gain of 2 kg or more, swelling, dizziness, faintness, palpitations, chest pain or breathlessness is a cardiovascular stop with no dose reduction, same-day referral for the cardiac symptoms and

	routine referral otherwise, and no restart without a prescriber review. ● Women need a ferritin of 30 micrograms/L or more and a normal thyroid stimulating hormone from the last 12 months, NHS or private, before a first supply; women of childbearing potential are defined in the standard way and the accepted contraceptive methods are listed. Frontal fibrosing alopecia, polycystic ovary syndrome and hyperandrogenism are named exclusions. Appendix 2 sets out the differential diagnosis the pharmacist must apply, and baseline and repeat photographs are specified. ● GP registration and notification within 7 days are conditions of supply. The stop-and-seek-help symptoms are on the label, in the counselling, in the written record given to the patient and in the follow-up row.

Appendix 1: Key references

- Loniten 2.5 mg tablets, Summary of Product Characteristics, Pfizer Limited, PL 00057/1006, text revised February 2026, electronic Medicines Compendium (the source of the pericardial effusion, ECG change and older-patient statements in this document).
- Akiska Y, Vano-Galvan S, et al. Low-dose oral minoxidil initiation for patients with hair loss: an international modified Delphi consensus statement. JAMA Dermatology, published online 20 November 2024.
- Vano-Galvan S, et al. Safety of low-dose oral minoxidil for hair loss: a multicenter study of 1,404 patients. J Am Acad Dermatol 2021;84:1644-51.
- Penha MA, et al. Oral minoxidil vs topical minoxidil for male androgenetic alopecia: a randomized clinical trial. JAMA Dermatology 2024;160:600-5.
- NICE Clinical Knowledge Summary: Alopecia, androgenetic, male; and Alopecia, androgenetic, female. Current versions.
- NICE NG136, Hypertension in adults: diagnosis and management, for the blood pressure thresholds used.
- British National Formulary, minoxidil, current edition.
- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions, including the provisions on off-label use, https://www.nice.org.uk/guidance/mpg2
- The Human Medicines Regulations 2012, Schedule 16, particulars to be included in a PGD.

Appendix 2: Is it androgenetic alopecia? What to look for

- ANDROGENETIC ALOPECIA (supply where the rest of the PGD allows): gradual onset over months or years; men: recession at the temples and thinning at the crown; women: widening of the central parting and thinning over the crown with the frontal hairline kept; scalp looks normal with the hair parted; follicular openings present; gentle pull test releases no more than a few hairs; no scale, redness, pustules, pain or itch.
- ALOPECIA AREATA (refer): one or more smooth round or oval patches of complete loss, sometimes with short "exclamation mark" hairs at the edge; may involve eyebrows, beard or nails. Onset over days to weeks.
- TELOGEN EFFLUVIUM (refer): diffuse shedding all over the scalp, handfuls of hair on washing or brushing, positive pull test, typically 2 to 4 months after childbirth, a serious illness, an operation, rapid weight loss, iron deficiency, thyroid disease or a new medicine. Usually recovers; needs the cause found.
- FRONTAL FIBROSING ALOPECIA (refer; most often women over 40): band-like recession of the frontal and temporal hairline, pale smooth skin behind the new hairline with the follicular openings gone, loss of the eyebrows, sometimes redness or scale around remaining hairs. Easily mistaken for female pattern loss; the preserved frontal hairline of true female pattern loss is the difference.
- OTHER SCARRING ALOPECIA (refer): patches where the skin is smooth and shiny with no follicular openings, often with redness, scale, pustules, pain or itch.
- TINEA CAPITIS (refer): scaly patches with broken hairs, sometimes swollen boggy areas, mostly in children but seen in adults; may be contagious.
- TRACTION ALOPECIA (refer): thinning along the margins where hair is pulled by tight styles, braids or extensions.

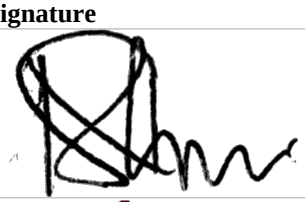
Appendix 3: Consent statement signed by the patient

- The consultation record carries the following statement above the patient's signature: "I have been told that minoxidil tablets are licensed in the UK for severe high blood pressure and not for hair loss, that this supply is off-label, why it is being used, what the alternatives are, that my blood pressure, pulse and weight will be checked at every supply, the symptoms for which I must stop and seek help, that extra body or facial hair is the commonest side effect, that a doctor must review me after six supplies, and (women) that I must not become pregnant while taking it. I consent to treatment on that basis and to my GP being informed."
- HYPERANDROGENISM IN A WOMAN (refer): hair loss with hirsutism, new or worsening acne, irregular or absent periods, a deepening voice, or a diagnosis of polycystic ovary syndrome. Needs assessment, and the hypertrichosis of minoxidil is a particular problem for these women.

Version and change record
Version: v002
Issued: 30 September 2026
Supersedes: Version 001, 28 September 2026
Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Signed on behalf of Get Real Health

Version v002, 30 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.